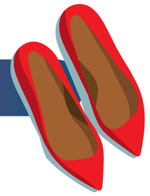
All serious incidents have an impact

Engage
Empower
Safeguard

Start here



Consider

Has the incident caused, or could the incident reasonably be expected to have caused, a consumer physical and/or psychological harm and/or discomfort? [Impacts]

If a consumer requires medical or psychological treatment from an incident, it's classed as a Priority 1 reportable incident.



What is meant by 'reasonably expected'?

Even if the incident does not appear to have caused harm to the consumer IDENTIFY and ASSESS impacts that are likely or probable.

Other considerations

Non-verbal or cognitive impairment

Observation is critical. Speak to members of the usual care team and significant others to understand any changes away from the person's normal. May need to observe over several days and update report as required. Provide alternative means of communication.



Diversity

Aboriginal and Torres Strait Islanders, care leavers, culturally and linguistically diverse people, people with disabilities, financially or socially disadvantaged people, homeless people, LGBTIQ+ people, parents of forced adoption or removed children, rural and regional consumers, veterans.

Know the person and history – how might the impact be different?

Prior trauma

For some people who have experienced trauma recently OR in the past the impact can be more severe. How will this be considered?

Physical assessment

Signs of injury, function, vital signs, pain assessment, x-ray, neurological observations and imaging.

Ongoing assessment is required as physical impacts are not always immediately evident.

Possible impacts

Pain, bleeding, bruising, cuts, fractures, soft tissue injury, dislocations, abrasions, scratches, joint injury, self-harm, sleep disturbance, skin reactions including sunburn and decreased conscious state.

Psychological assessment

Instead of asking "Are you OK?" use open-ended questions such as:



- What are you feeling after the incident?
- How can we help you recover from the incident?
- Observe – is the person's behaviour different from their norm?
- Ask significant others and staff if they have noticed changes.

Possible impacts

Agitation, withdrawal, isolation, depression, anxiety, fear, embarrassment, shame, sadness or distress, refusal of care, hyper vigilance, seeking constant reassurance, silence, reduced fluid and food intake.

Physical and psychological impacts may be immediate or delayed for hours, days or weeks.

Now consider common impacts related to specific SIRS incidents

These are not exhaustive – there will be others

Unreasonable use of force

Physical: bruising, skin tears, swelling, fractured or dislocated limb, pain, head strike, black eye, nosebleed, split lip

Psychological: fear, mistrust, withdrawal, teariness



Know the person

Stealing or financial coercion by a staff member

Physical: inability to afford health items, comforts or social care etc.

Psychological: distrust, refusal of care, embarrassment, shame

Unlawful sexual contact or inappropriate sexual conduct

Priority 1 always

Physical: STIs, genital or anal trauma, pain

Psychological: shame, fear, relationship disruption



Neglect

Physical: untreated medical condition, weight loss, deconditioning, incontinence, pressure injury

Psychological: spiritual or cultural disconnection, loneliness, boredom, fear, worthlessness

Psychological or emotional abuse

Physical: fear response

Psychological: difficulty controlling emotions and/or maintaining relationships, resistance to care, anger, withdrawal

Walk in their shoes



Inappropriate use of restrictive practices

Physical: deconditioning, delirium, falls, pressure injuries, chafing

Psychological: social isolation, depression, boredom, loss of autonomy

Unexpected death

Priority 1 always

Physical: symptoms of medical condition leading up to death

Psychological: any distress experienced leading up to death

How would you feel if this happened to you?

Unexplained absence

Priority 1 always

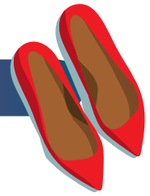
Physical: any injury or discomfort received leaving or while away from care.

Psychological: fear, wariness, distress, confusion, embarrassment

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Australian Government
Aged Care Quality and Safety Commission

1800 951 822
agedcarequality.gov.au